

Pre-Travel Health Advisory

United States · 7 days · Leisure

Departing 5 December 2026

Traveller: Dike Wisdom

Low risk

COMPLETED

Apr 24, 2026

TRIP AT A GLANCE	
DURATION	7 days
PURPOSE	Leisure
TRAVELLING	Alone
ACCOMMODATION	Hotel (urban)
INSURANCE	Yes

ITINERARY-SPECIFIC GUIDANCE		
Trip type: ONE_WAY		
This is a one-way trip departing Nairobi, Kenya on 5 December 2026 and arriving in Oklahoma, United States. The 7-day duration spans two distinct health risk environments: Nairobi (medium risk, rabies-endemic, malaria-present, food and water precautions required) and Oklahoma (low risk, developed healthcare infrastructure). The highest-risk phase of this trip is the Nairobi departure phase and any activities conducted in Kenya before or around the departure date.		
ROUTE GUIDANCE BY STOP		
STOP	COUNTRY	GUIDANCE
Oklahoma	United States	Oklahoma, United States is a low health risk destination with excellent healthcare infrastructure. Standard travel health precautions apply. December weather is cold; dress appropriately in layers. Road safety standards are high; drive on the right. Tap water is safe to drink. Healthcare is readily accessible but can be expensive; ensure your travel insurance provides comprehensive medical cover including emergency treatment in the United States, which has high healthcare costs. Be aware of the 9-hour time zone difference from Nairobi (EAT to CST); allow several days to adjust. If you experience any fever, gastrointestinal illness, or other symptoms after arrival, inform your treating clinician of your recent travel to Kenya.

HEALTH RISK OVERVIEW		
CATEGORY	LEVEL	SUMMARY
Food and water safety	MODERATE	Nairobi carries a moderate food and water safety risk. Tap water is generally not recommended for drinking. Oklahoma, United States carries low food and water risk with treated municipal water supplies. Strict food and water hygiene should be observed during the Nairobi transit or departure phase.

Vector-borne diseases	MODERATE	Nairobi city itself sits at approximately 1,700 metres elevation and carries a lower malaria risk than rural Kenya, though risk is not zero. Oklahoma has no significant vector-borne disease risk beyond low-level West Nile virus activity in summer months. Standard mosquito precautions apply for the Nairobi phase.
Respiratory infections	MODERATE	Influenza and COVID-19 circulate year-round in both Kenya and the United States. Nairobi has urban air quality concerns. Volunteering and farm or animal contact activities increase exposure to respiratory pathogens. Standard respiratory hygiene is recommended throughout.
Environmental health (heat, sun, air quality)	MODERATE	Nairobi has moderate UV exposure and urban air pollution. Oklahoma in December is generally cool to cold with low UV risk. Sun protection remains relevant during outdoor hiking activities. Air quality in Oklahoma City is generally acceptable.
Injuries and road traffic safety	MODERATE	Road traffic injury is a leading cause of travel-related harm in Kenya. Nairobi traffic is dense and road safety standards differ from Western norms. Oklahoma road conditions are generally good. Hiking and outdoor activities carry inherent injury risk. Seatbelts and reputable transport providers are strongly advised.
Rabies and animal contact	HIGH	Kenya is rabies-endemic. Dike has indicated farm and animal contact as a planned activity. This combination significantly elevates rabies exposure risk. Pre-exposure rabies prophylaxis is strongly recommended before departure. Immediate wound care and access to post-exposure prophylaxis at destination must be planned in advance. Oklahoma, United States has rabies present in wildlife (bats, raccoons, skunks) but post-exposure treatment is readily available.
Blood-borne and sexual health	HIGH	Dike has indicated anticipated casual relationships, inconsistent barrier protection (sometimes), a prior STI history, and acknowledged substance use affecting adherence. Kenya has a significant HIV prevalence. This combination represents a high blood-borne and sexual health risk. Consistent condom use, hepatitis B vaccination, and PrEP (pre-exposure prophylaxis for HIV) discussion are strongly recommended.
Altitude-related illness	LOW	Nairobi sits at approximately 1,700 metres, which is below the 2,500 metre threshold for acute mountain sickness risk. Oklahoma is at low elevation. No altitude-related illness risk applies to this itinerary.

FLIGHT & JOURNEY HEALTH

VTE Risk Level: **MEDIUM**

PREVENTION MEASURES

- The longest flight leg is 6 hours, which places this in the medium VTE (deep vein thrombosis) risk category
- Wear graduated compression stockings (15 to 30 mmHg) for the duration of the flight
- Perform in-seat leg exercises every 30 to 60 minutes: ankle circles, heel raises, and calf pumps
- Walk the aisle for 5 minutes every 1 to 2 hours when the seatbelt sign is off
- Stay well hydrated throughout the flight; avoid excessive alcohol and caffeine which promote dehydration

▪ Avoid sleeping in positions that compress the legs for extended periods	
▪ No additional pharmacological VTE prophylaxis is indicated at this time given no other VTE risk factors reported	
MEDICATION TIMING GUIDANCE	No current medications reported. No insulin or time-sensitive medication timing adjustments required. If malaria prophylaxis (atovaquone-proguanil) is prescribed, confirm the start date with your clinician; it is typically started 1 to 2 days before entering a malaria-risk area.

VACCINATIONS		
VACCINE	STATUS	GUIDANCE
Routine immunizations (e.g. MMR, varicella, dTdap, polio /IPV)	Believed up to date	Recommended Dike believes his routine vaccinations are up to date. Confirm documentation before travel. Ensure tetanus-diphtheria-pertussis (Tdap) booster is current, particularly given planned farm, animal, and outdoor activities where wound risk is elevated. A booster is recommended if more than 5 years have elapsed. Polio vaccination should be confirmed as Kenya is a polio-vigilant country. Verify MMR 2-dose series is documented.
Influenza	Unknown	Recommended Annual influenza vaccination is recommended for all international travellers. December travel coincides with influenza season in the Northern Hemisphere. Administer before departure if not received in the past 12 months.
COVID-19	Unknown	Recommended Ensure COVID-19 vaccination is up to date with the most current booster available. Check United States entry requirements for any COVID-19 documentation at the time of travel. Kenya has no current mandatory COVID-19 entry requirement but this should be verified closer to departure.
Hepatitis A	Not received (no travel vaccines reported)	Recommended Hepatitis A vaccination is recommended for travel to Kenya. Transmission occurs via contaminated food and water. As Dike has not received any travel-related vaccines previously, this should be administered as a priority. A single dose provides protection for up to 1 year; a booster at 6 to 12 months provides long-term immunity. Administer at least 2 weeks before departure if possible.
Hepatitis B	Not confirmed	Strongly recommended Hepatitis B vaccination is strongly recommended given Dike's indicated risk behaviours including casual sexual relationships with inconsistent barrier protection and a prior STI history. Kenya has high hepatitis B prevalence. An accelerated schedule (doses at day 0, day 7, and day 21 with a booster at 12 months) is available if time before departure is limited. Discuss with your clinician urgently.
Typhoid	Not received	Recommended

		Typhoid vaccination is recommended for travel to Kenya, where typhoid risk is documented by CDC and WHO. Either the oral live attenuated vaccine (Ty21a, 4 capsules over 7 days) or the injectable Vi polysaccharide vaccine (single dose) is appropriate. The injectable vaccine should be administered at least 2 weeks before departure. Note: typhoid vaccination is not required or recommended for Oklahoma, United States.
Yellow fever	Not received	Review required Kenya does not require yellow fever vaccination for entry unless the traveller is arriving from a yellow fever endemic country. Dike is departing from Nairobi, Kenya to the United States. The United States does not require yellow fever vaccination for entry. However, if Dike's travel history in the preceding weeks included any yellow fever endemic countries, a valid yellow fever certificate may be required. Confirm your full travel history with your clinician. Yellow fever vaccination is not routinely recommended for Nairobi city travel.
Japanese encephalitis	Not applicable	Not indicated Japanese encephalitis is not present in Kenya or the United States. This vaccine is not indicated for this itinerary.
Meningococcal	Unknown	Consider Kenya falls within the sub-Saharan meningococcal belt. Meningococcal ACWY vaccination is worth discussing with your clinician, particularly given planned volunteering and humanitarian activities involving close contact with local populations. This is not a mandatory requirement for this itinerary but is a reasonable precaution.
Rabies pre-exposure	Not received	Strongly recommended Rabies pre-exposure prophylaxis is strongly recommended. Kenya is rabies-endemic and Dike has indicated farm and animal contact as a planned activity. Pre-exposure prophylaxis consists of 3 doses administered on days 0, 7, and 21 (or 28). This does not eliminate the need for post-exposure treatment if a bite or scratch occurs, but it simplifies the post-exposure regimen and buys critical time in remote settings. Given the 7-day trip duration, the full pre-exposure course must be started well in advance of departure. Discuss urgently with your travel medicine clinician.
Cholera (oral vaccine)	Not received	Low priority for this itinerary Cholera vaccination (Dukoral or Vaxchora) is not routinely recommended for urban hotel-based travel to Nairobi. It may be considered if Dike plans to spend significant time in areas with poor sanitation or if the volunteering activities involve humanitarian work in high-risk settings. Discuss with your clinician based on the specific nature of the volunteering work.

MALARIA PREVENTION

Risk Level: **LOW**

Atovaquone-proguanil (Malarone) preferred if prophylaxis is indicated

Nairobi city itself sits at approximately 1,700 metres elevation and is generally

RECOMMENDED CHEMOPROPHYLAXIS	considered low malaria risk compared to rural Kenya. However, malaria risk in Kenya is not zero, and the CDC advises that travellers to Kenya should take antimalarial precautions. Given the short 7-day stay, urban hotel accommodation, and departure from Nairobi, the risk is at the lower end. However, if Dike's activities take him outside Nairobi city or into peri-urban areas, risk increases. Atovaquone-proguanil (Malarone) is the preferred agent: it is well tolerated, has no neuropsychiatric side effects (relevant given substance use history), requires only 1 to 2 days of post-travel dosing, and is safe without G6PD testing. Doxycycline is an alternative but requires strict sun protection and daily adherence. Mefloquine is not recommended given Dike's indicated alcohol and substance use. Discuss with your clinician whether prophylaxis is warranted for your specific Nairobi activities.
MOSQUITO PROTECTION MEASURES	
▪ Apply DEET-based insect repellent (30 to 50 percent concentration) to all exposed skin, especially at dawn and dusk	
▪ Wear long-sleeved shirts and long trousers in the evenings and in outdoor settings	
▪ Sleep in air-conditioned rooms or use a permethrin-treated bed net if ventilation is open	
▪ Use permethrin spray on clothing and gear for outdoor and farm activities	
▪ Avoid standing water and areas with high mosquito density, particularly after rainfall	
CONTRAINDICATIONS / ALTERNATIVES	Mefloquine is not recommended given anticipated alcohol consumption and substance use, which increase neuropsychiatric side effect risk. Primaquine and tafenoquine require G6PD testing before use and are not first-line for this itinerary. Doxycycline requires strict daily adherence and sun protection.

MEDICAL ADVICE & RECOMMENDATIONS	
TOPIC	DETAILS
Pre-travel review and vaccination records	Dike has not received any prior travel-related vaccines and has not previously visited a travel clinic. A comprehensive pre-travel consultation with a travel medicine clinician is strongly recommended before 5 December 2026. Priority vaccines to discuss include hepatitis A, hepatitis B (accelerated schedule), typhoid, rabies pre-exposure prophylaxis, and Tdap booster. Confirm routine vaccination documentation. Allow sufficient lead time: the rabies pre-exposure course requires at least 21 days to complete. Book your travel clinic appointment immediately.
Food and water hygiene	In Nairobi, drink only bottled or boiled water. Avoid ice in drinks unless confirmed to be made from purified water. Eat at reputable establishments; avoid raw or undercooked meat, shellfish, and street food from unverified vendors. Wash hands thoroughly before eating and after using the toilet. Carry alcohol-based hand sanitiser (at least 60 percent alcohol). In Oklahoma, United States, tap water is safe and food hygiene standards are high; standard precautions apply.
Vector bite prevention	Apply DEET repellent (30 to 50 percent) to exposed skin when outdoors in Nairobi, particularly during dawn and dusk hours when Anopheles mosquitoes are most active. Wear light-coloured, long-sleeved clothing during outdoor and farm activities. Use permethrin-treated clothing and

	gear for hiking and farm contact. Ensure accommodation has screened windows or use a bed net. In Oklahoma in December, mosquito activity is minimal; standard precautions suffice.
Sun, heat, and environmental precautions	Nairobi has moderate UV radiation year-round despite its equatorial location. Apply SPF 30 or higher broad-spectrum sunscreen during outdoor activities, particularly hiking. Wear a hat and UV-protective sunglasses. Stay hydrated in the Nairobi heat. Oklahoma in December is typically cold; dress in layers and be prepared for cold weather conditions. Air quality in Nairobi can be poor in urban areas; those with respiratory sensitivities should carry any prescribed inhalers.
Injury and road safety	Road traffic accidents are a leading cause of injury and death for travellers in Kenya. Always wear a seatbelt. Use reputable, licensed taxi or ride-share services (e.g. Uber, Bolt) rather than informal transport. Avoid travelling by road at night where possible. During hiking and outdoor activities, stay on marked trails, wear appropriate footwear, and carry a basic first aid kit. Inform someone of your planned route and expected return time when hiking alone. In Oklahoma, standard road safety rules apply; drive on the right.
Sexual health and blood exposure	Dike has indicated anticipated casual sexual relationships, inconsistent condom use (sometimes), and a prior STI history. Kenya has a high HIV prevalence (approximately 4 to 5 percent in the general adult population) and significant rates of other STIs. Consistent and correct condom use with every sexual encounter is strongly recommended. Discuss PrEP (pre-exposure prophylaxis for HIV prevention) with your clinician before travel; PrEP is highly effective when taken as prescribed and is appropriate given the indicated risk profile. Ensure hepatitis B vaccination is completed before departure. If a potential HIV exposure occurs, PEP (post-exposure prophylaxis) must be started within 72 hours; identify a clinic in Nairobi that can provide PEP before you travel. Avoid tattooing, piercing, or any invasive cosmetic procedures abroad unless sterile professional facilities are confirmed. Do not share needles or personal items that may carry blood.
Jet lag, sleep, and mental wellbeing	The time difference between Nairobi (EAT, UTC+3) and Oklahoma (CST, UTC-6) is 9 hours. This is a significant time zone shift for a 7-day trip. Adjust sleep and meal times gradually in the days before departure. On arrival in Oklahoma, expose yourself to natural daylight to help reset your circadian rhythm. Avoid excessive alcohol use to manage jet lag, as alcohol disrupts sleep quality and impairs immune function. Travelling alone can be isolating; maintain regular contact with family or friends. If you experience significant anxiety, mood changes, or difficulty coping, seek support early.
Malaria and other chemoprophylaxis (state if not indicated)	Malaria prophylaxis should be discussed with your clinician. Nairobi city carries a low but non-zero malaria risk. If your activities remain strictly within Nairobi city and urban hotel settings, your clinician may determine prophylaxis is not essential. However, if any activities take you to peri-urban or rural areas of Kenya, prophylaxis is recommended. Atovaquone-proguanil (Malarone) is the preferred agent for this itinerary given its tolerability, short post-travel dosing requirement, and absence of neuropsychiatric side effects. Mefloquine is not recommended given anticipated alcohol and substance use. No malaria prophylaxis is required for Oklahoma, United States.
Traveller-specific	Several factors specific to Dike's health context require direct attention.

considerations (from health context)	First, Dike has no prior travel vaccine history and has never visited a travel clinic; a comprehensive pre-travel consultation is urgent given the departure date of 5 December 2026. Second, planned farm and animal contact in Kenya, a rabies-endemic country, makes rabies pre-exposure prophylaxis a high priority requiring immediate action as the 3-dose course takes a minimum of 21 days. Third, the combination of casual sexual relationships, inconsistent barrier protection, prior STI history, and alcohol and substance use creates a compounding high-risk profile for HIV, hepatitis B, and other STIs; PrEP discussion and hepatitis B vaccination are urgent priorities. Fourth, volunteering and humanitarian activities may involve close contact with vulnerable populations and potential exposure to bloodborne pathogens; hepatitis B status must be confirmed. Fifth, substance use affecting adherence is flagged; discuss medication adherence strategies with your clinician, particularly for any prophylaxis prescribed.
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MEDICATION LOGISTICS	
PACKAGING	Carry all medications and vaccines (if any cold-chain items are prescribed) in original manufacturer packaging with pharmacy labels intact. Carry a physician letter listing all prescribed medications by generic and brand name.
SUPPLY RULE	Carry a minimum of double the required supply split between carry-on and checked luggage to protect against loss or delay.
DESTINATION LEGALITY VERIFIED	Yes — confirm all medications are legal at destination.

CLINICAL DECISION FLAGS
▪ TREE_3_MEDIUM_RISK_DESTINATION
▪ TREE_4_DURATION
▪ TREE_5_FLIGHT
▪ TREE_9_ANIMAL_FARM_CONTACT_RABIES_ENDEMIC_DESTINATION
▪ TREE_9_VOLUNTEERING_HUMANITARIAN_BLOODBORNE_RISK
▪ TREE_14_CASUAL_SEXUAL_RELATIONSHIPS_INCONSISTENT_BARRIER_PROTECTION
▪ TREE_14_STI_HISTORY
▪ TREE_14_ALCOHOL_AND_SUBSTANCE_USE_ADHERENCE_RISK
▪ TREE_12_NO_PRIOR_TRAVEL_VACCINES
▪ TREE_13_NO_INTERNATIONAL_TRAVEL_LAST_12_MONTHS

CONTRAINDICATIONS (MEDICATIONS & VACCINES)
▪ Mefloquine is contraindicated given anticipated alcohol consumption and substance use affecting adherence (neuropsychiatric risk)
▪ No other active contraindications identified

SPECIALIST REFERRALS

CONDITION	SPECIALIST	URGENCY
No prior travel vaccines and multiple high-priority vaccines required including rabies pre-exposure prophylaxis (3-dose course), hepatitis A, hepatitis B accelerated schedule, and typhoid	Travel medicine clinician or travel health clinic	BEFORE_TRAVEL
High HIV and STI risk profile: casual relationships, inconsistent barrier protection, prior STI history, alcohol and substance use	Sexual health clinician or GP for PrEP assessment and hepatitis B vaccination	BEFORE_TRAVEL
Planned farm and animal contact in rabies-endemic Kenya with no prior rabies vaccination	Travel medicine clinician for rabies pre-exposure prophylaxis initiation	BEFORE_TRAVEL
Volunteering and humanitarian activities with potential bloodborne pathogen exposure	Occupational health or travel medicine clinician for hepatitis B status confirmation and universal precautions briefing	BEFORE_TRAVEL

SEXUAL HEALTH & RISK BEHAVIOURS

Risk Level: HIGH

PREVENTION ADVICE

- Use condoms consistently and correctly with every sexual encounter, including oral sex where appropriate
- Discuss PrEP (pre-exposure prophylaxis for HIV) with your clinician before departure; PrEP is highly effective at preventing HIV when taken as prescribed and is appropriate given your risk profile
- Ensure hepatitis B vaccination is completed before travel; Kenya has high hepatitis B prevalence
- Identify a clinic in Nairobi that can provide PEP (post-exposure prophylaxis for HIV) before you travel; PEP must be started within 72 hours of a potential HIV exposure
- Be aware that alcohol and substance use impair judgment and increase the likelihood of unprotected sex; plan ahead and carry condoms at all times
- Avoid tattooing, piercing, or invasive cosmetic procedures abroad unless sterile professional facilities are confirmed
- Arrange a post-travel STI screening test within 4 weeks of return, particularly if any unprotected sexual contact occurred

PREP/PEP

PrEP/PEP availability discussed for high-risk destination.

AFTER YOU RETURN

RED FLAGS — SEEK IMMEDIATE CARE IF YOU EXPERIENCE

Seek emergency medical care immediately if you develop: high fever (above 38 degrees Celsius) at any point within 3 months of return from Kenya (possible malaria), difficulty breathing, chest pain, severe headache with neck stiffness, any animal bite or scratch that was not treated with post-exposure rabies prophylaxis, or signs of a severe allergic reaction to any vaccine or medication received.

WITHIN 1 WEEK

- Monitor for fever, chills, or flu-like symptoms; any fever within 3 months of travel to Kenya must be treated as possible malaria until proven otherwise and requires urgent medical assessment
- Report any fever to your GP or emergency department immediately and inform them of your travel to Kenya

WITHIN 4 WEEKS

- Attend a post-travel STI screening appointment if any unprotected sexual contact occurred during the trip
- Monitor for gastrointestinal symptoms including prolonged diarrhoea, nausea, or abdominal pain; report to your GP if symptoms persist beyond 3 days
- Watch for any skin rashes, unusual bites, or wounds that are not healing normally, particularly following farm or animal contact
- If you received any animal bites or scratches in Kenya and did not complete post-exposure rabies treatment, seek emergency medical care immediately regardless of how minor the wound appeared

BEYOND 4 WEEKS

- Complete the hepatitis B vaccination booster dose at 12 months if the accelerated schedule was started before travel
- Complete the hepatitis A booster dose at 6 to 12 months after the initial dose for long-term immunity
- If you started PrEP before travel, continue as directed by your clinician and attend follow-up appointments for kidney function and STI monitoring
- Consider a general post-travel health check with your GP if you feel unwell or have any concerns about exposures during the trip

MEDICAL CARE & CONTACTS**CLINICS & FACILITIES**

NAME	LOCATION & NOTES
Aga Khan University Hospital Nairobi	3rd Parklands Avenue, Nairobi, Kenya +254 20 366 2000 · Central Nairobi Major international-standard hospital with 24-hour emergency services, travel medicine, and post-exposure rabies treatment availability
Nairobi Hospital	Argwings Kodhek Road, Nairobi, Kenya +254 20 284 5000 · Central Nairobi Full-service private hospital with emergency department and infectious disease services
OU Health University of Oklahoma Medical Center	700 NE 13th Street, Oklahoma City, OK 73104, United States +1 405 271 4700 · Oklahoma City Major academic medical centre with full emergency and infectious disease services

EMBASSY & CONSULAR

NAME	DETAILS

Nigerian High Commission, Nairobi, Kenya	Lenana Road, Hurlingham, Nairobi, Kenya. Tel: +254 20 271 4899
Nigerian Embassy, Washington DC (consular services for United States)	3519 International Court NW, Washington DC 20008, United States. Tel: +1 202 986 8400
EMERGENCY NUMBERS	
LABEL	CONTACT
Kenya Emergency Services	999 or 112
Kenya Police	999
United States Emergency Services	911
AMREF Flying Doctors (Kenya medical evacuation)	+254 20 699 2000

NEXT STEPS
▪ Book a travel medicine clinic appointment immediately; the rabies pre-exposure prophylaxis course requires a minimum of 21 days to complete and must be started without delay before 5 December 2026 ▪ Discuss and initiate hepatitis B accelerated vaccination schedule (days 0, 7, and 21) at your travel clinic appointment ▪ Receive hepatitis A and typhoid vaccinations at your travel clinic appointment ▪ Discuss PrEP for HIV prevention with your GP or sexual health clinician before departure ▪ Confirm Tdap booster and routine vaccination documentation with your GP ▪ Discuss malaria prophylaxis with your clinician; atovaquone-proguanil (Malarone) is the preferred agent if prophylaxis is recommended for your specific Nairobi activities ▪ Purchase graduated compression stockings (15 to 30 mmHg) for your 6-hour flight ▪ Identify and note the address of a clinic in Nairobi that provides PEP (HIV post-exposure prophylaxis) before you travel ▪ Ensure your travel insurance policy covers medical evacuation, emergency treatment in the United States (high cost environment), and pre-existing or travel-acquired conditions ▪ Carry condoms for the entire trip and plan ahead for situations where alcohol may impair judgment

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DOCTOR VERIFICATION & DIGITAL SIGNATURE

This travel health plan has been reviewed and approved by a licensed physician on the TMAG Doctor Network.

PHYSICIAN NAME	Mat Smith
LICENSE NUMBER	MT898320084
VALIDATION DATE	24 April 2026
STATUS	TMAG Verified & Approved



Digital Signature

TMAG Verified Seal

This document was generated by Travel Medicine Advisory Global and approved by a licensed physician.